

PATIENT CLINICAL NARRATIVE RECORDS — CONFIDENTIAL All patient information in this document is simulated for demonstration purposes only and does not represent real clinical data.

CLINICAL NARRATIVE NOTE PT-101 | 5 January 2025

Patient Identification & Background The patient registered under PT-101 is a 38-year-old female, married, and working as a graphic designer, who was registered under the unique patient identifier PT-101. She belongs to the O+ blood group and has a prior reaction to penicillin presenting with rash. The patient is a non-smoker and consumes alcohol socially. She presented to the outpatient department on 5 January 2025 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness The patient's primary reason for the visit today was intermittent palpitations and mild chest tightness over the past month. She has a known background of poorly controlled blood pressure with home readings consistently above 145/90 mmHg, along with episodes of dizziness on standing. The symptoms have been present and progressing over recent weeks, prompting her to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses The patient carries an established diagnosis of Essential Hypertension and Orthostatic Hypotension, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of her conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements On examination today, vital signs were recorded as follows: blood pressure was 142/88 mmHg, heart rate was 88 beats per minute, respiratory rate was 18 breaths per minute, and body temperature was 98.6°F. Oxygen saturation on room air was 98%. The patient's recorded weight was 68 kg with a height of 162 cm, yielding a body mass index of 25.9 kg/m², which is noted for ongoing monitoring.

Physical Examination On general inspection, the patient was conscious, cooperative, and oriented to time, place, and person. She appeared comfortable at rest. Cardiovascular examination revealed a regular rate and rhythm with no added sounds. Respiratory examination was clear bilaterally. Abdomen was soft and non-tender. Pertinent examination findings relevant to the presenting diagnosis included a postural drop in blood pressure of 22 mmHg systolic on standing. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings Relevant investigations reviewed at this visit demonstrated 24-hour ambulatory blood pressure monitoring showing a non-dipping pattern and ECG within normal

limits. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications The patient is currently prescribed the following medications: Amlodipine 5 mg once daily, Telmisartan 40 mg once daily, Vitamin D3 60,000 IU weekly, and Multivitamin tablet once daily. Medication reconciliation was performed at today's visit, and the patient confirmed adherence to the prescribed regimen. She was counselled regarding the importance of not self-discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment The clinical evaluation today suggests that PT-101 remains showing gradual improvement with appropriate management. Lab results and examination findings are broadly consistent with the documented diagnoses. The patient appears highly adherent and engaged in self-care. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Essential Hypertension and Orthostatic Hypotension. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, the dose of the primary medication has been uptitrated in view of suboptimal control. Furthermore, dietary and lifestyle counselling was reinforced at this visit, with specific advice on hydration and gradual postural changes. She was also informed that annual screening investigations have been ordered as per protocol. Finally, the patient is to be reviewed in six weeks and instructed to seek earlier review if any new symptoms arise. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 5 January 2025 | PT-101

CLINICAL NARRATIVE NOTE PT-102 | 4 February 2025

Patient Identification & Background The patient registered under PT-102 is a 29-year-old male, unmarried, and working as a delivery rider, who was registered under the unique patient identifier PT-102. He belongs to the B+ blood group and has a prior reaction to peanuts presenting with anaphylaxis. The patient is a non-smoker and does not consume alcohol. He presented to the outpatient department on 4 February 2025 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness The patient's primary reason for the visit today was recurrent lower back pain radiating to the right leg, worsened by prolonged sitting. He has a known background of intermittent numbness and tingling in the right foot for the past four months. The symptoms have been present and

progressing over recent weeks, prompting him to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses The patient carries an established diagnosis of Lumbar Disc Herniation (L4-L5) and Sciatica, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of his conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements On examination today, vital signs were recorded as follows: blood pressure was 124/78 mmHg, heart rate was 74 beats per minute, respiratory rate was 16 breaths per minute, and body temperature was 98.3°F. Oxygen saturation on room air was 99%. The patient's recorded weight was 70 kg with a height of 175 cm, yielding a body mass index of 22.9 kg/m², which is noted for ongoing monitoring.

Physical Examination On general inspection, the patient was conscious, cooperative, and oriented to time, place, and person. He appeared in mild distress while changing positions. Cardiovascular examination revealed a regular rate and rhythm with no added sounds. Respiratory examination was clear bilaterally. Abdomen was soft and non-tender. Pertinent examination findings relevant to the presenting diagnosis included a positive straight leg raise test on the right side at 40 degrees with diminished ankle reflex. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings Relevant investigations reviewed at this visit demonstrated MRI lumbar spine showing a posterolateral disc herniation at L4-L5 with mild nerve root compression. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications The patient is currently prescribed the following medications: Naproxen 500 mg twice daily after meals, Gabapentin 100 mg at bedtime, Methylcobalamin 1500 mcg once daily, and a topical diclofenac gel applied twice daily. Medication reconciliation was performed at today's visit, and the patient confirmed adherence to the prescribed regimen. He was counselled regarding the importance of not self-discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment The clinical evaluation today suggests that PT-102 remains clinically stable with no significant deterioration. Lab results and examination findings are broadly consistent with the documented diagnoses. The patient appears moderately adherent and engaged in self-care. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Lumbar Disc Herniation (L4-L5) and Sciatica. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, current medications are maintained without changes as the patient remains well controlled on conservative therapy. Furthermore, a structured physiotherapy programme has been initiated for symptomatic relief. He was also informed that ergonomic advice regarding posture and lifting techniques was provided. Finally, the patient is to be reviewed in four weeks and instructed to seek earlier review if any new symptoms arise, particularly bladder or bowel disturbance. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 4 February 2025 | PT-102

CLINICAL NARRATIVE NOTE PT-103 | 6 March 2025

Patient Identification & Background The patient registered under PT-103 is a 71-year-old male, widowed, and working as a retired teacher, who was registered under the unique patient identifier PT-103. He belongs to the O- blood group and has a prior reaction to contrast dye presenting with facial swelling. The patient is an ex-smoker who quit fifteen years ago with no current alcohol use. He presented to the outpatient department on 6 March 2025 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness The patient's primary reason for the visit today was increasing forgetfulness and difficulty recalling recent conversations over the past six months. He has a known background of mild gait instability and two recent falls without loss of consciousness. The symptoms have been present and progressing over recent weeks, prompting him to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses The patient carries an established diagnosis of Mild Cognitive Impairment and Type 2 Diabetes Mellitus, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of his conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements On examination today, vital signs were recorded as follows: blood pressure was 132/80 mmHg, heart rate was 70 beats per minute, respiratory rate was 17 breaths per minute, and body temperature was 98.1°F. Oxygen saturation on room air was 97%. The patient's recorded weight was 75 kg with a height of 169 cm, yielding a body mass index of 26.3 kg/m², which is noted for ongoing monitoring.

Physical Examination On general inspection, the patient was conscious and cooperative but appeared mildly anxious about his memory concerns. Cardiovascular examination revealed a regular rate and rhythm with no added sounds. Respiratory examination was clear bilaterally. Abdomen was soft and non-tender. Pertinent examination findings relevant to the presenting diagnosis included a Mini-Mental State Examination score of 24/30 with mild deficits in delayed recall, and a mildly widened-based gait. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings Relevant investigations reviewed at this visit demonstrated HbA1c of 7.8% and a non-contrast MRI brain showing mild age-related cortical atrophy without acute findings. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications The patient is currently prescribed the following medications: Metformin 1000 mg twice daily with meals, Glimepiride 2 mg once daily before breakfast, Donepezil 5 mg at bedtime, and Calcium with Vitamin D supplement once daily. Medication reconciliation was performed at today's visit, and the patient confirmed adherence to the prescribed regimen. He was counselled regarding the importance of not self-discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment On today's assessment, PT-103 demonstrates some deterioration requiring attention since the last visit, particularly with respect to cognitive function and fall risk. The clinical picture is consistent with the underlying diagnoses, and the patient has been moderately adherent to the prescribed treatment plan. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Mild Cognitive Impairment and Type 2 Diabetes Mellitus. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, a new medication has been added to the existing regimen after discussing the rationale and potential side effects with the patient and his family. Furthermore, a home safety assessment and falls-prevention referral has been initiated. He was also informed that a formal neuropsychological assessment has been arranged. Finally,

follow-up in six weeks with repeat biochemical parameters and reassessment of cognitive status. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 6 March 2025 | PT-103

CLINICAL NARRATIVE NOTE PT-104 | 5 April 2025

Patient Identification & Background The patient registered under PT-104 is a 33-year-old female, married, and working as a primary school teacher, who was registered under the unique patient identifier PT-104. She belongs to the B- blood group and has a prior reaction to shellfish presenting with hives and lip swelling. The patient is a non-smoker and does not consume alcohol. She presented to the outpatient department on 5 April 2025 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness The patient's primary reason for the visit today was persistent fatigue, hair thinning, and unexplained weight gain of approximately 5 kg over four months. She has a known background of irregular menstrual cycles over the past year with cycle lengths ranging from 35 to 50 days. The symptoms have been present and progressing over recent weeks, prompting her to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses The patient carries an established diagnosis of Subclinical Hypothyroidism and Polycystic Ovary Syndrome, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of her conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements On examination today, vital signs were recorded as follows: blood pressure was 120/76 mmHg, heart rate was 78 beats per minute, respiratory rate was 16 breaths per minute, and body temperature was 98.0°F. Oxygen saturation on room air was 99%. The patient's recorded weight was 79 kg with a height of 158 cm, yielding a body mass index of 31.6 kg/m², which is noted for ongoing monitoring.

Physical Examination On general inspection, the patient was conscious, cooperative, and oriented to time, place, and person. She appeared well but mildly fatigued. Cardiovascular examination revealed a regular rate and rhythm with no added sounds. Respiratory examination was clear bilaterally. Abdomen was soft and non-tender. Pertinent examination findings relevant to the presenting diagnosis included mild diffuse thinning of scalp hair and mild acanthosis nigricans noted over the posterior neck. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains.

Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings Relevant investigations reviewed at this visit demonstrated TSH of 6.2 mIU/L with normal free T4, and a pelvic ultrasound showing bilateral polycystic ovarian morphology. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications The patient is currently prescribed the following medications: Metformin 500 mg once daily with the evening meal, Inositol 2 g once daily, Folic acid 5 mg once daily, and a combined oral contraceptive pill for cycle regulation. Medication reconciliation was performed at today's visit, and the patient confirmed adherence to the prescribed regimen. She was counselled regarding the importance of not self-discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment On today's assessment, PT-104 is managing well overall despite the chronicity of the underlying conditions. There is marginal but encouraging progress since the last visit, and the patient has been well adherent and motivated regarding medication compliance and lifestyle recommendations. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Subclinical Hypothyroidism and Polycystic Ovary Syndrome. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, the dose of the primary metformin therapy has been uptitrated in view of suboptimal weight and metabolic control. Furthermore, a dietitian referral has been initiated to support a structured weight-management plan. She was also informed that repeat thyroid function testing has been ordered in three months to reassess the need for levothyroxine. Finally, the patient is to return in three months or sooner if menstrual irregularities worsen significantly. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.

Attending Physician | Date: 5 April 2025 | PT-104

CLINICAL NARRATIVE NOTE PT-105 | 4 May 2025

Patient Identification & Background The patient registered under PT-105 is a 46-year-old male, married, and working as a long-distance truck driver, who was registered under the unique patient identifier

PT-105. He belongs to the AB+ blood group and has a prior reaction to tramadol presenting with severe nausea and vomiting. The patient is a current smoker of approximately fifteen cigarettes per day with regular alcohol consumption on weekends. He presented to the outpatient department on 4 May 2025 for evaluation and ongoing management.

Chief Complaint & History of Presenting Illness The patient's primary reason for the visit today was loud snoring witnessed by his spouse along with excessive daytime sleepiness and morning headaches. He has a known background of difficulty staying awake while driving on long trips over the past year. The symptoms have been present and progressing over recent weeks, prompting him to seek medical attention. There is no history of recent hospitalisation, surgical procedures, or significant changes in the living environment. The family history is non-contributory for any new conditions. A thorough systems review was conducted and was otherwise unremarkable beyond the presenting complaints.

Past Medical History & Active Diagnoses The patient carries an established diagnosis of Obstructive Sleep Apnoea and Metabolic Syndrome, both of which are currently being managed on an outpatient basis. The diagnoses were confirmed through a combination of clinical evaluation, laboratory investigations, and relevant imaging studies in prior visits. The patient has been under regular follow-up and understands the chronic nature of his conditions. There are no prior surgical histories of significance. Immunisation history is up to date as per the national schedule.

Vital Signs & Anthropometric Measurements On examination today, vital signs were recorded as follows: blood pressure was 138/88 mmHg, heart rate was 82 beats per minute, respiratory rate was 18 breaths per minute, and body temperature was 98.5°F. Oxygen saturation on room air was 95%. The patient's recorded weight was 102 kg with a height of 176 cm, yielding a body mass index of 32.9 kg/m², which is noted for ongoing monitoring.

Physical Examination On general inspection, the patient was conscious, cooperative, but appeared visibly fatigued with periorbital puffiness. Cardiovascular examination revealed a regular rate and rhythm with no added sounds. Respiratory examination was clear bilaterally. Abdomen was soft, non-tender, with increased waist circumference noted. Pertinent examination findings relevant to the presenting diagnosis included a Mallampati score of III, a neck circumference of 43 cm, and a calculated Epworth Sleepiness Scale score of 16. No lymphadenopathy was detected on palpation of the cervical, axillary, or inguinal chains. Cranial nerve examination was grossly normal. Skin and mucous membranes were well hydrated with no significant abnormalities observed.

Investigations & Diagnostic Findings Relevant investigations reviewed at this visit demonstrated overnight polysomnography showing an apnoea-hypopnoea index of 34 events per hour, consistent with severe obstructive sleep apnoea, alongside fasting triglycerides of 240 mg/dL. These results were correlated with the patient's clinical presentation and compared with prior records to track longitudinal trends. Additional investigation results including complete blood count, liver function tests, thyroid function, and urine routine examination were reviewed where available. Imaging reports and specialist correspondence, if any, were also noted and factored into the management decision.

Current Medications The patient is currently prescribed the following medications: CPAP therapy at 12 cmH2O nightly, Atorvastatin 10 mg at bedtime, Metformin 500 mg once daily with dinner, and Omega-3 fatty acid supplement once daily. Medication reconciliation was performed at today's visit, and the patient confirmed adherence to the prescribed regimen. He was counselled regarding the importance of not self-discontinuing medications, reporting any adverse effects promptly, and maintaining refill schedules. Drug interactions were reviewed and no new clinically significant interactions were identified at this time.

Assessment Overall, the patient's condition appears to be showing gradual improvement with appropriate management. The clinical picture is consistent with the underlying diagnoses, and the patient demonstrates occasionally non-compliant with CPAP use due to discomfort with the mask. The combination of current symptomatology, examination findings, and recent investigations supports a continued diagnosis of Obstructive Sleep Apnoea and Metabolic Syndrome. No new acute medical issues were identified at today's visit that would require immediate escalation of care.

Management Plan & Follow-Up Following thorough evaluation, the management plan was discussed in detail with the patient. In the first instance, the CPAP mask interface has been changed and a refitting appointment has been arranged to improve tolerance. Furthermore, urgent referral to the relevant specialist for occupational driving fitness assessment has been initiated. He was also informed that patient education regarding warning signs requiring emergency consultation, including chest pain and severe breathlessness, was provided. Finally, repeat polysomnography and metabolic panel have been ordered and the patient is to return in eight weeks for review. The patient verbally acknowledged understanding of the management plan, expressed willingness to comply, and was reassured regarding the overall prognosis with appropriate lifestyle modification and medication adherence. Emergency contact information and instructions for when to seek urgent review were clearly communicated.